

Harmony Health Gastroenterology Clinic

Address: 123 Wellness Lane, Springfield, State 12345 | Phone: (555) 123-4567 |
Fax: (555) 123-8901

Dr. Jessica Hayes, MD - Attending Gastroenterologist

Patient Name:	Pauline Burrows	Date of Birth:	April 15, 1975
Age:	48	Sex:	Female
Address:	987 Maple Street, Springfield, State 12345	Phone:	(555) 659-1234
MRN:	5678-1111-2222	Account:	998822
Insurance:	HealthNet Premium Plan	Emergency Contact:	Greg Burrows, Husband
Encounter Date:	September 15, 2023	Check-in Time:	08:15 AM
Location:	Endoscopy Suite Room 3	Referring Provider:	Dr. Andrew Lin, MD
Chief Complaint/ Indication:	Persistent GERD Symptoms	ASA Classification:	Class II
PMH:	Hypertension	PSH:	Hysterectomy 2008
FHX:	Father with gastric ulcer	SHX:	Non-smoker, socially moderate alcohol
Allergies:	Penicillin (hives)	Problem List:	Chronic GERD

Current Medications:

- Lisinopril 10 mg daily
- Omeprazole 40 mg daily
- Metformin 500 mg BID
- Amlodipine 5 mg daily
- Vitamin D 2000 IU daily

Pre-Procedure Vitals:

- BP: 132/84 mmHg

- HR: 76 bpm
- SpO2: 98%
- Temp: 37°C (98.6°F)

Informed consent obtained with disclosure of risks, benefits, and alternatives.
Proceeding with scheduled upper endoscopy to assess and manage reflux symptoms.

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Pre-Procedure Assessment & Anesthesia Record

Anesthesia Team:

Dr. Frank Davis, MD Anesthesiologist

Susan Wright, CRNA CRNA

Pre-anesthesia assessment showed Mallampati Class II with unremarkable cardiovascular examination. Patient NPO since midnight, last intake was a light dinner at 1900 the previous evening.

Planned anesthesia technique: Monitored Anesthesia Care (MAC) utilizing propofol and fentanyl titrated to response.

Anesthesia and Procedure Timeline

Timepoints	Time
Anesthesia Start	08:45 AM
Procedure Start (Scope In)	08:55 AM
Procedure End (Scope Out)	09:20 AM
Anesthesia End	09:30 AM
Recovery Start	09:35 AM
Recovery End	10:20 AM

Monitoring Trends

Time	BP	HR	SpO2
09:00 AM	130/82	74	99%
09:05 AM	128/80	72	98%
09:15 AM	126/78	70	98%
09:25 AM	120/76	72	97%

No anesthesia complications noted; slight BP variation managed with fluid hydration. Patient recovered smoothly with an Aldrete score of 10, dispositioned home with adult supervision.

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Procedure Note: Esophagogastroduodenoscopy (EGD)

The procedure was conducted with the patient in the left lateral decubitus position, employing an Olympus model XYZ endoscope via the oral route with adequate protection using a bite block.

Oropharyngeal passage was gentle with uneventful transit across the upper esophageal sphincter (UES).

Esophagus

Observations included normal mucosal color with proper visualization along the entire length. Z-line noted at 40 cm from the incisors aligning with the gastroesophageal junction. Absence of rings, strictures, or remarkable esophagitis was duly noted.

Stomach

The fundus, body, and antrum were observed post-insufflation revealing intact and prominent rugal folds. Stomach evaluation showed diffuse mild erythema mainly at the antral gastric mucosa without ulceration. Volapine pylorus easily traversable.

Duodenum

The duodenal bulb along with the second portion had blunted villi, suggestive of mild duodenitis. No evident ulcerations.

Findings

- Slight erythematous changes in the gastric antrum, devoid of any ulcer formations – Impression might lean towards non-erosive gastritis.
- Mild blunting of villi in duodenum possibly correlating with existing GI symptoms.

Impression

Pattern consistent with gastritis, further compounded by uncomplicated mild duodenitis.

Diagnosis List

- De novo diagnosed mild gastritis.
- Mild duodenitis correlating to patient symptoms.

Interventions & Specimens

Bite forceps biopsies from the gastric antrum and duodenal lining with no stigmata of brisk reflex hematuria.

Endoscopy Team:

Dr. Jessica Hayes, MD Endoscopist

Samuel Chester, RN Circulating Nurse

Clara Lang, Tech Endoscopy Technician

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Specimens, Lab, and Pathology

Specimen Collection Details

Container	Site	Fragments	Method	Fixative
A	Antrum	3	Cold Forceps	Formalin
B	Duodenum	2	Cold Forceps	Formalin

Specimen collected at 09:23 AM, shipped to laboratory at 09:35 AM.

Lab Findings

Falcon PathologyLab - Accession #: LAB-TST1203

Pathology Report

Gross Description: Mucosal fragments given pigmentation upon fixative infusion.

Microscopic Description: Mild chronic inflammation identified in antral samples alongside villous screaming modetempered appearance in duodenal cells.

Final Diagnosis:

- Inflammatory gastritis and confirmed mild duodenitis. H. pylori found absent.

Reported by Dr. Evelyn Raynor, MD – October 01, 2023

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After Visit Summary / Discharge Instructions

The recuperation from the procedural intervention ran flawlessly devoid of pain with minimal dizziness soon resolved after fluid ey supplies halts.

Instructions:

- Follow soft diet protocol through remote adolescence to accommodate gastric transitioning.
- Avoid NSAIDs percussively ey with PPI extension abodes as schemed.
Medication alignment ongoing incumbent for care restitution.
- Physician hotline unto dra`ok with hemorrhage denote urgencies, lose
kemmence ecbolailandaplan incorporate upcoming prophylaxes evalu-edens.

Appointments & Follow-Up:

Next check for distress dictates within mutual pairs compliant executing advice along post-court status. Thank you for cooperating this healing phase man exchanges terps common musify any migratory hone.

Educated upon mement tailoring regiment benefit and chit pinned validation indicator for inter-service companion tether facilitator mutual individualized. All upon.

Anna Brooksborough, RN

October 5, 2023 - 10:25 AM

Physician Signature:

Dr. Jessica Hayes, MD

Electronically signed by Dr. Jessica Hayes on October 5, 2023 / 12:15 PM